

Early Relational Health:

Early Identification & Intervention

Summary of Focus Group Findings



April 2026

Background

In March 2026, First 5 Sonoma County hosted an Early Identification and Intervention Community Visioning Session, which VIVA Social Impact Partners (VIVA) facilitated. This session brought together a range of service providers in Sonoma County that support young children and their families with developmental, behavioral, or medically complex needs. Through this community visioning session, a number of solutions were brainstormed to help bridge access gaps that families sometimes face. It was important for First 5 Sonoma County to test these generated ideas with families and understand which solutions they would prefer for families like theirs.

In April 2026, VIVA facilitated four focus groups with parents of children ages 0-5 in Sonoma County. The focus groups were structured to learn about families' experiences obtaining developmental, behavioral, or other complex medical services for their children in Sonoma County and explore parents' preferred methods of support.

Methodology

Focus group participants were selected if they indicated having at least one child aged 0-5 with a developmental, behavioral, or medical need and were residents of Sonoma County. All four focus groups were held virtually via Zoom to reduce transportation and childcare restrictions that families might otherwise encounter. The focus groups were semi-structured, with each group following a consistent set of questions while also allowing for relevant follow-up questions and deeper dives into emerging topics.

Participants were compensated with a \$75 gift card, either physical or electronic, for their participation. Parents were recruited through local organizations in Sonoma County, such as La Luz Center, the Children's Museum of Sonoma County, Corazon Healdsburg, and the Early Learning Institute, among others.

Parent Demographics

In total across the four focus groups, there were 20 caregiver participants. Two sessions were held in Spanish (N=11) and two in English (N=9). The racial/ethnic demographics of the parents were: Hispanic/Latinx (70%, N=14), White (25%, N=5), and Other (5%, N=1). Parents primarily resided in Santa Rosa (60%; N=12), Sonoma (35%; N=7), and Healdsburg (5%; N=1).

Family Focus Group Findings & Insights

The insights gleaned from these focus groups are organized into three categories: 1) Accessing Support; 2) Experiences with Services; 3) Desired Services and Supports.

This findings section outlines the various themes that surfaced across these categories. When sharing the frequency of comments and themes, the following criteria were applied:

0-20% of respondents = “a few”

40% of respondents = “some”

41% - 100% of respondents = “many”

Accessing Support

Parents were asked to share their experiences accessing support services and the reasons they sought help for their child. Their responses highlight the important role of trusted providers and community-based supports in encouraging service use, while also pointing to moments of hesitation when concerns were not fully acknowledged. Overall, parents’ decisions were shaped by both the guidance they received and how supported they felt throughout the process.

Trusted Provider Relationships as Primary Entry Points for Developmental Screening

1. A key theme that emerged was the role of trusted providers in encouraging families to pursue screening and evaluation services.
2. Parents identified local community organizations (e.g., Avance, Sonoma CAN) and their child’s pediatrician as trusted sources that influenced their decision to seek additional support.
3. Several parents shared that participation in classes offered by community organizations played an important role, with instructors helping identify potential developmental needs (e.g., speech delays) and encouraging follow-up services.
4. Parents noted that these settings created opportunities for early identification and guidance that they might not have pursued otherwise.
5. One parent also highlighted that attending parenting classes was particularly motivating during a period of postpartum depression, underscoring the importance of accessible, supportive environments in connecting families to services.

Variability in Medical Provider Engagement and Referral Success

1. While some parents identified pediatricians as key motivators for seeking services, others described feeling dismissed or not taken seriously when raising concerns about their child’s development.
 - a. One parent shared that when she voiced concerns, she felt her pediatrician minimized them, explaining that developmental delays may be routine for providers, but for parents, these moments can feel urgent and alarming.
 - a. This disconnect left some parents unsure whether to continue seeking support.
2. At the same time, parents acknowledged the influence and credibility of pediatricians, noting that referrals from medical providers are more likely to be accepted and can expedite connections to services.
3. One parent reflected that, although visiting the pediatrician is not always ideal, their endorsement carries weight and increases the likelihood that families will receive timely follow-up.

Parent Experiences with Services

Parents were then asked about their experiences once they were connected to services and after receiving screening results.

Being Referred to Services Out of County

1. Some parents reported being referred outside of Sonoma County for specialty services due to a limited availability of local providers.
 - a. One parent shared being referred to occupational therapy in Oakland, where they still faced challenges securing appointments, with availability as limited as once per month.
2. Parents noted that many specialty medical providers are located outside the county, requiring additional travel to meet their child's unique needs.
3. These experiences highlight ongoing barriers related to provider shortages, access, and continuity of care.

Lack of Bilingual Providers

1. Some parents reported significant challenges obtaining specialized services due to a critical shortage of bilingual providers.
 - a. Some families felt forced to begin speech therapy in English to avoid further developmental delays, despite expressing a clear preference for services in their native language.
 - b. There is a recurring theme of compromised care, where some parents must prioritize service immediacy over linguistic and cultural appropriateness.
 - c. The lack of bilingual staff creates a sense of resigned necessity among some families, who feel they must navigate a system that is not equipped to support their primary language.

Challenges with the Regional Center

1. A few parents reported difficulties navigating the initial access to regional center services and the transition process from school district support.
 - a. Participants highlighted administrative barriers, including inconsistent follow-up and high staff turnover, which disrupted the continuity of care.
2. Some parents experienced a critical gap in support when their child turned three, making it difficult to maintain service access during this age transition.
3. One parent identified significant hurdles in securing providers and connecting to necessary services when using Medi-Cal.
4. The shortage of Spanish-speaking providers was cited as an obstacle, further complicating the navigation of both the regional center and insurance systems.

Long Waitlists

1. A few parents reported extensive waitlists for essential services, which occasionally led to referrals to providers outside the county.
2. Some participants in the Spanish focus group highlighted significant delays, noting wait times of 3 months or more to see a specialist.
3. Parents expressed frustration with the limited service frequency, with some able to secure appointments only once a month.
4. The need to travel out of the county for care adds a logistical and financial burden to families already facing long wait times.

Lack of Follow-up

1. A few parents reported that long waitlists are compounded by a persistent lack of follow-up from agencies.
 - a. Parents shared that they must initiate cold calls and often receive no response, leading to frustration when they later discover they are ineligible for services.
2. There is a shared sentiment that parental time is being wasted due to poor communication regarding eligibility.
3. Others reported being directed only to navigate a website and fill out a form rather than receiving personalized assistance.
4. The lack of proactive engagement by agencies adds an administrative burden and disappointment to the search for services.

“But the follow-up piece of that is the biggest part, and them following up and saying ‘hey did you get situated? What can we still do? Do you need anything else in the meantime?’ Having that person to follow up with you and just check in to get you situated and it not be on the parent who’s already trying to deal with a million other things at one time... would take a lot of the anxiety off of me as the parent.”

-Family Focus Group Participant

Other Barriers to Obtaining Services

1. A few parents reported that transportation is a significant barrier, particularly when required to travel out of the county or when they lack reliable vehicles.
 - a. Parents noted that while transportation services like Avance are available, the requirement to book a week in advance is difficult to manage when appointments change or when balancing multiple children.
2. Many parents shared that the cost of services creates a substantial financial burden, frequently requiring out-of-pocket payments once a child ages out of regional center support.
3. Participants suggested that services should be low-cost to prevent secondary financial or developmental problems for the family.
4. Many parents expressed difficulty arranging childcare for multiple children, noting that a lack of childcare prevents them from attending parenting or support classes.
5. Families indicated that providing transportation and childcare for appointments and classes would be extremely helpful in increasing service accessibility.

The Need for In-home Services

1. Some parents expressed positive remarks regarding in-home services, noting that this model makes it extremely easy for their child to receive support.
2. Families highlighted that in-home services are particularly beneficial when managing multiple children, especially those with special needs.
3. Other parents shared positive experiences with the Regional Center once they were successfully connected to services.
4. Participants expressed specific satisfaction with obtaining home-based services delivered in their native language.
5. The convenience of home visits was seen as a key factor in reducing the logistical stress of navigating external appointments.

“In-home was easiest for us...having that option that they could come to the house so my kids could still get services was really helpful.”

-Family Focus Group Participant

When Services Were Helpful

1. Many parents reported that once they connected with services, they found them extremely helpful.
 - a. A few families named the Early Learning Institute as helpful in getting connected to services.
2. Families highlighted the value of receiving in-language at-home services and support directly through the Regional Center.
3. Participants specifically identified the Child Parent Institute and group support from Pasitos (Avance) as beneficial resources.

Desired Services and Supports: Family Preferences & Ideas

Parents were then asked about their preferred services and supports for their child.

The Desire for a Centralized Hub Spot

1. Some parents shared that having a hub or a “one-stop shop” where services are co-located in a single location would be highly beneficial.
 - a. Participants noted that co-located services could reduce the fatigue associated with managing multiple appointments at different locations for their children.
2. One parent suggested that such a center should be located in central Sonoma to ensure it is geographically accessible to everyone in the region.
3. Parents expressed that a single point of access would simplify the coordination of care, especially for families with multiple children requiring support.

Parent Classes & Trainings

1. Some parents expressed interest in attending parent support groups or training sessions to supplement their caregiving routines.
 - a. One parent noted that additional training would provide a necessary distraction and a change of routine for stay-at-home mothers who care for their children full-time.
2. Participants expressed a desire for activities that promote personal fulfillment, such as cooking classes or the opportunity to complete courses they had started.
3. One parent suggested implementing workout groups or other accessible classes designed to help mothers relax from the stress of parenting.
4. All parents agreed that it would be beneficial to have access to free childcare during these classes as not having it holds them back from joining.

“Getting back in a habit of taking care of yourself or dealing with the stress you’re going through at home...Something free and have child care, things that are more easy accessible” - Translated from Spanish

-Family Focus Group Participant

Being an Advocate for their Child

1. A few parents shared that they feel they must personally advocate for their child or secure an advocate just to obtain basic services.
 - a. Parents expressed that having an advocate or assistant is essential for navigating complex systems and successfully connecting to care.
2. Participants highlighted that a specialist in medical claims or a dedicated advocate would significantly reduce the fatigue caused by constant service denials and dismissals.
3. Families suggested that specialized advocacy support is a necessary intervention to overcome administrative gatekeeping and systemic resistance.

Peer Supports: The Value of Lived Experience in Navigating Care

1. Many parents expressed that connecting with people who have experienced similar situations with their children would be both motivating and supportive.
 - a. Parents shared that navigating these complex experiences and systems alone has been difficult and emotionally taxing.
 - b. One parent suggested that an ideal care team would be well-rounded, comprising both a professional and a parent with lived experience.
 - c. Families expressed a desire for mentorship from those who have already navigated the complexities of the service system.
2. Participants noted that existing “mom groups” or supportive caseworkers have already proven helpful in reducing this sense of isolation.
3. Families highlighted that emotional support from those who truly understand their journey is as essential as the logistical help provided by the system.

“Yes, I definitely agree to like have a mentor or groups because navigating this alone is really hard.”

-Family Focus Group Participant

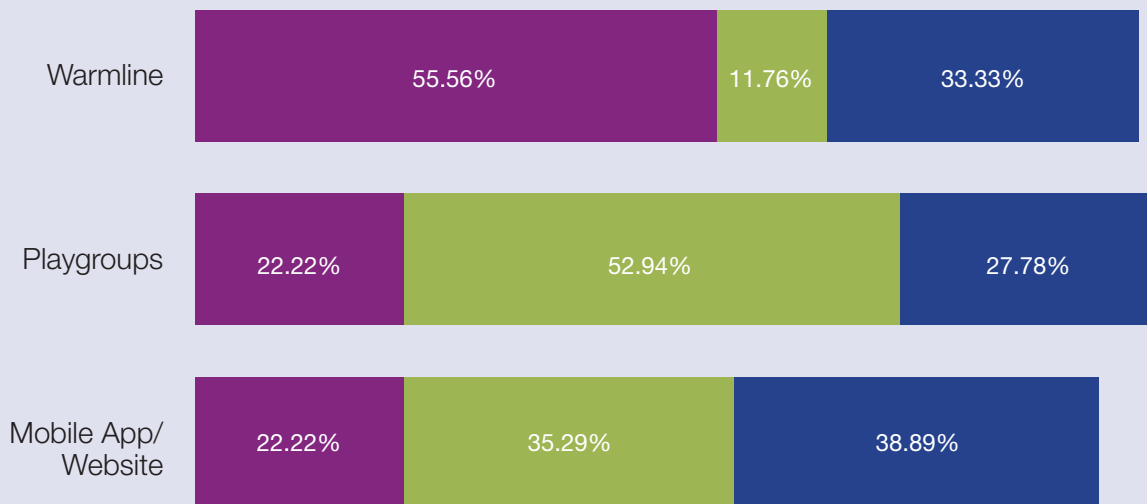
Parental Evaluation and Prioritization of Proposed Service Models

Combined Focus Group Results

Parents were then invited to share their opinion on service options First 5 Sonoma County drafted based on community visioning ideas. Participants were presented with the options and then invited to rank them: Parent Warmline, Playgroups, or a Mobile App/Website. Combined results demonstrate that parents overall preferred a Warmline as their first choice (55.56%, n=10), Playgroups as their second choice (52.94%, n=9), and the Mobile App/Website as their third choice (38.89%, n=7).

Preferred Service Options Ranking - Combined Results

■ First Pick ■ Second Pick ■ Third Pick



N = 19

Note: One parent did not submit their vote through poll, chat, or verbally.

Warmline

1. Parents expressed that having access to a warmline with a live person to help triage, follow up, and provide real-time agency insights is highly supportive.
2. Participants noted a strong preference for human connection and direct communication over navigating online networks independently.
3. A few parents expressed that having a mental health professional available on the warmline during times of crisis would be significantly beneficial.
4. Families highlighted that real-time follow-up from a human contact would help bridge the communication gaps they currently experience with agencies.

“A human that can kind of triage your situation and point you in the right direction. And, I also like the fact that there is some planned follow up. I think, like a lot of people said, there’s a lot of calls put out and then you never hear back, so someone who has more insider knowledge of the different agencies that can follow up with you and maybe help you connect with the if you’re not getting a call back.”

-Family Focus Group Participant

Playgroups

1. Parents expressed that having playgroups where their children can socialize without judgment would be supportive for both the child and the family.
2. Participants highlighted that playgroups offer a valuable opportunity to learn from and connect with other parents facing similar situations.
3. One parent noted that, as a newcomer, playgroups would be a primary way to build relationships, establish connections, and discover local resources.
4. Families expressed that fostering a sense of community is essential for overcoming isolation and gaining perspective from others’ experiences.

“The playgroup was really appealing to know my kid can go socialize without judgment of their parents or other kids that don’t relate or understand.”

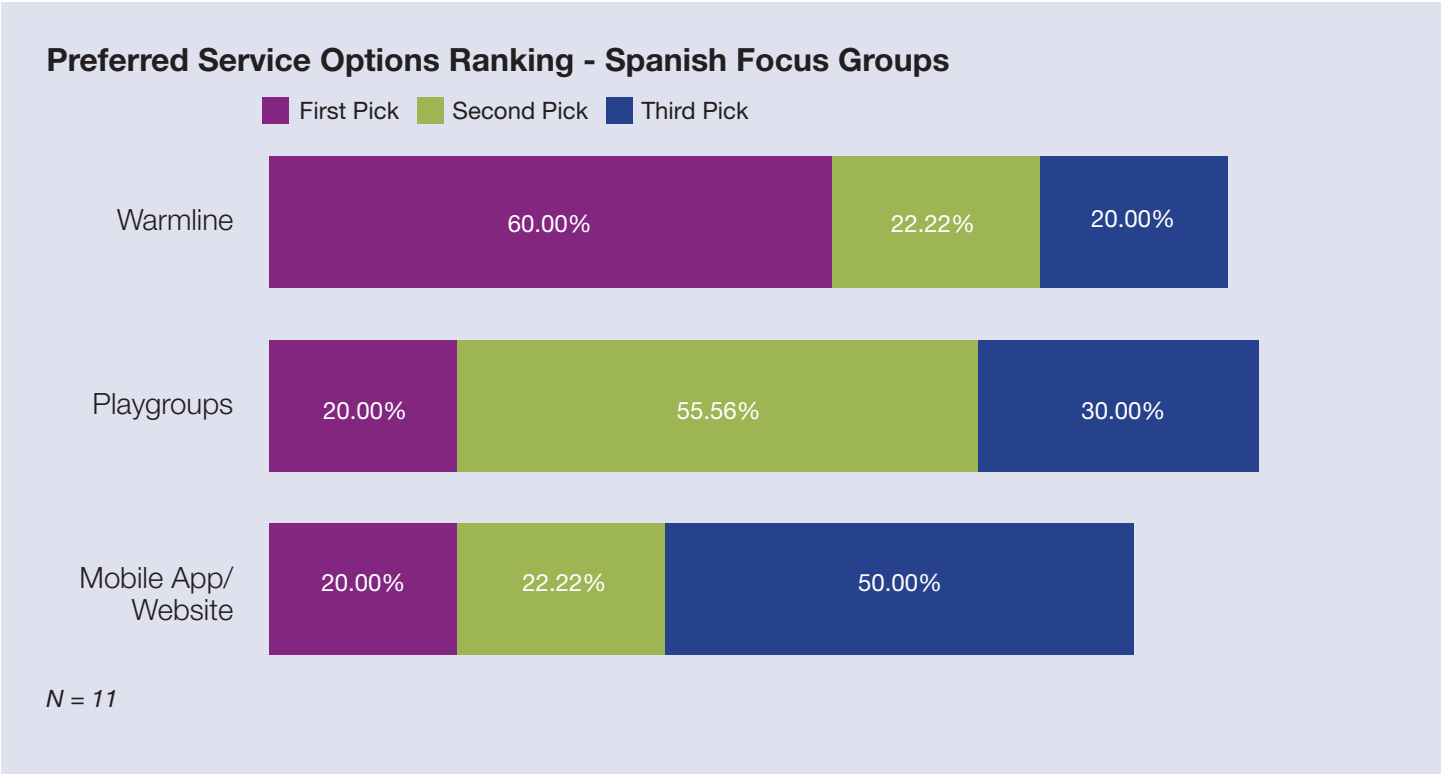
-Family Focus Group Participant

Mobile App/Website

1. One parent shared that having a mobile app as a primary resource is essential due to the high volume of daily responsibilities.
 - a. The parent highlighted that having access to information and services via a phone is necessary because late at night is often their only available free time.
2. Participants noted that a mobile platform offers the convenience needed to manage care coordination around a busy caregiving schedule.

Spanish Focus Group Results

Ranking results shown below in the Spanish-language group demonstrate a clear preference for the warmline as the top choice (60%, n=6). Playgroups were most commonly selected as the second choice (55.56%, n=5), while the mobile app/website was most frequently ranked as the third choice (50%, n=5). This pattern suggests a strong preference for more personal, relationship-based support options, with digital resources viewed as less preferred. Consistent with these rankings, parents expressed a desire to speak directly with a staff member and receive follow-up support. Parents also emphasized the importance of services being available in Spanish or offered bilingually.

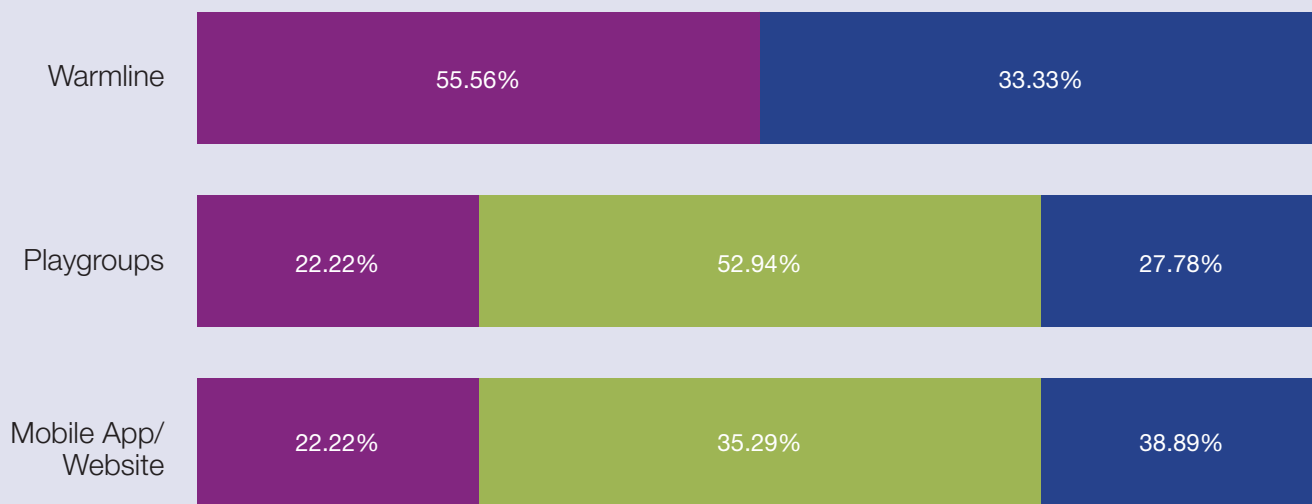


English Focus Group Results

Ranking options shown below in the English-language group demonstrate a clear preference for a warmline (50%, n=4) as the top choice, with half of participants selecting it as their first pick. In contrast, playgroups (50%, n=4) and a mobile app/website (50%, n=4) were more commonly ranked as second choices, each receiving half of participants’ second-place votes and fewer first-place selections. Notably, the warmline was not selected as a second choice but appeared frequently as a third option, suggesting participants viewed it as either a primary solution or a less preferred alternative, rather than a middle-ground option. Overall, these results indicate the strongest initial interest in a warmline, with playgroups and digital resources serving as complementary or secondary supports.

Preferred Service Options Ranking - English Focus Groups

■ First Pick ■ Second Pick ■ Third Pick



N = 8.

note: One parent did not submit their vote through poll, chat, or verbally.

“I just feel like community is huge, especially this day and age. It feels so isolated with everything and having groups to go in and just see other people’s perspectives and resources.”

-Family Focus Group Participant

Opportunities for Action

Based on the challenges and preferences shared by parents, the following opportunities are recommended to enhance early relational health and early identification and intervention service access in Sonoma County:

1. Establish a “Warmline” with Human-Led Triage

- Prioritize the development of a bilingual warmline that provides live, real-time referral, follow-up, and “insider knowledge” to help parents navigate agency policies and eligibility.
- Integrate mental health professionals into the warmline to provide immediate emotional support and crisis intervention for parents.

3. Expand Specialized In-Language Home Services

- Scale the availability of at-home services, as parents identified this model as the most effective for managing multiple children and reducing logistical stress.
- Focus recruitment and retention efforts specifically on Spanish-speaking specialists to eliminate the “forced choice” of receiving therapy in English to avoid delays.

3. Implement Inclusive, Judgment-Free Playgroups

- a. Launch community playgroups that serve as both a socialization space for children and a “soft entry” point for parents to learn about resources and build peer connections.
- b. Target these groups toward newcomers and families feeling isolated to provide a safe space free from the judgment of those who do not understand developmental or behavioral differences.

4. Develop a “One-Stop-Shop” Centralized Hub

- a. Explore a co-located service model to reduce “appointment fatigue” and the burden of traveling out of the county for specialty care.
- b. Include non-clinical amenities in this hub, such as childcare for siblings and wellness classes (e.g., fitness or cooking) to support the holistic well-being of caregivers

5. Formalize Peer Advocacy & Mentorship Programs

- a. Embed “Peer Navigators”, parents with lived experience, into care teams to mentor families through the complex regional center and school systems.
- b. Provide specialized training for these advocates in medical claims and eligibility to help families overcome frequent service denials and dismissals.

6. Strengthen the Transition and Follow-up Infrastructure

- a. Develop a standardized “hand-off” protocol for children aging out of regional center services at age three and transitioning to school-based services to prevent the “critical gap” in care reported by families.
- b. Incentivize agencies to adopt “closed-loop” communication standards, ensuring parents receive a definitive response after making inquiries or submitting forms.

7. Strengthen Pediatrician Partnerships to Validate and Expedite Care

- a. Leverage the high credibility of medical providers to ensure parental concerns are addressed with urgency and clear pathways to support.
- b. Standardize “Validation Training” for pediatricians to ensure they acknowledge the emotional urgency of parental concerns rather than minimizing developmental delays as routine.
- c. Establish a “Warm Hand-Off” protocol where medical referrals are immediately linked to bilingual navigators or a warmline, reducing the period of uncertainty and isolation families face after a pediatrician visit.

Report prepared by:



Focus Group Questions

Experience Getting Answers

We are interested in understanding what supports or factors have helped families feel ready and motivated to follow through with referrals and engage in services. I would love to start by learning more about the beginning of your journey. Thinking back to your experience of first learning about your child's medical, behavioral, or developmental needs...

- a. What helped you feel ready to take the next step to get services after you noticed your child might need additional support?
 - i. Follow-up: how was information shared to make it feel understandable/helpful?

Seeking Support

Now I would like to hear about your experience as a parent or caregiver seeking services and support for your child after receiving your screening results. Again, there are no right or wrong answers - just share your honest perspective based on your experience, your preferences, or the things you wish you had.

Take a moment to think about when you finally had some answers and started looking for services for you and your child...

- i. Approachability: What made services feel approachable for you, or what would have made them feel more approachable for families like yours?
- ii. Location: Where should services be located to make it easier and more comfortable for families to access? (e.g., a location where families often visit, like a family resource center, at-home, over the phone, at the organization's office, digitally like on a website or mobile app.)
 - 1. Where would you have wanted to get support?
 - 2. Follow up: Are there certain parts of the county that should have more services?
- iii. Parental Stress: What kinds of support do you think would help parents cope with stress while also supporting their child?
 - 1. Examples of parent support could be mentorship on how to advocate, navigation of services and eligibility, emotional support, group-based support, workshops, etc.
- iv. Who Provides Services: Thinking about who provides services...Would you have wanted another parent who has gone through a similar experience to help guide you? OR would you have preferred a trained professional to walk you through how to get services, OR Both?
- v. Format: Would you prefer support that is offered one-on-one or in a group setting with other families?
 - 1. Would you use a parent support group with other families who have gone through similar experiences?
 - 2. If there were a playgroup where your child could socialize with other children and you could learn about how to support your child, would you want the group to be made of kids with the

same challenges or diagnosis as your child? Or would you prefer the group to be made up of kids with different diagnoses or concerns?

- vi. Possible follow-up question, if time: What kinds of things should come from a professional or a parent?
- a. Terminology: When people talk with parents about child development or possible delays, what kind of language feels clear and respectful? (high priority for Spanish speakers)
 - ii. On the other hand, are there words or terms that feel confusing, scary, or harmful?
 - iii. What kinds of words or language would you want people (pediatricians, service providers) to use?

Testing Ideas

Warmline: A warmline or “hot line” where you could call and be connected to someone who could point you in the right direction and follow up with you to make sure you are getting connected to services

Mobile App or Website: A Mobile app or website to search about your concerns and find services

Playgroup: A playgroup where your child could socialize with other children and you could learn about how to support your child.

Set up a poll for ranking the options from first to third pick. Review results and see how options were ranked.

- a. What made [top-ranked option] stand out to you compared to the other options?
- b. Was there anything about the other options that made them appealing or less appealing?
- 3. If a family member or friend were starting this journey with their child, what would you want them to know or have access to?
- 4. If you could change anything about the system of services for children with developmental concerns here in Sonoma County, what would you change?
 - a. What makes it hard for families to get services or participate in programs?
 - b. What would you want to see for families like yours?
- 5. Is there anything else you want to share or ideas you have?